CPY 903

Advanced Assessment and Diagnosis

A Textbook in Counselling Psychology

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Advanced Assessment and Diagnosis

A Doctoral-Level Textbook in Counselling Psychology

[AUTHOR TO SUPPLY]

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Diagnostic criteria quoted or paraphrased in this book are drawn from the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision (American Psychiatric Association, 2022) and from the Clinical Descriptions and Diagnostic Requirements for ICD-11 Mental, Behavioural and Neurodevelopmental Disorders (World Health Organization, 2024). Copyright in those works remains with their respective owners.

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Abbreviations

Acronyms and abbreviations used in this book

Abbreviation	Expansion
AERA	American Educational Research Association
APA	American Psychological Association
ADHD	Attention-deficit/hyperactivity disorder
ASD	Autism spectrum disorder
AUDIT	Alcohol Use Disorders Identification Test
BDD	Body dysmorphic disorder
CFI	Cultural Formulation Interview
CI	Confidence interval
CBT	Cognitive behavioural therapy
DSM-5-TR	Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition, Text Revision
GAD-7	Generalised Anxiety Disorder-7 item scale
HCR-20	Historical-Clinical-Risk Management-20
ICD-11	International Classification of Diseases, Eleventh Revision
ICF	International Classification of Functioning, Disability and Health
IQ	Intelligence quotient
IRT	Item response theory
ITC	International Test Commission
MSE	Mental status examination
MMPI	Minnesota Multiphasic Personality Inventory
NPV	Negative predictive value
OCD	Obsessive-compulsive disorder
PAI	Personality Assessment Inventory
PHQ-9	Patient Health Questionnaire-9 item scale
PID-5	Personality Inventory for DSM-5
PPV	Positive predictive value
PTSD	Post-traumatic stress disorder

Abbreviation	Expansion
RIASEC	Realistic, Investigative, Artistic, Social, Enterprising, Conventional (Holland's model of vocational interests)
ROC	Receiver operating characteristic
SCID-5	Structured Clinical Interview for DSM-5 Disorders
SCID-5-AMPD	Structured Clinical Interview for the DSM-5 Alternative Model for Personality Disorders
SCID-5-PD	Structured Clinical Interview for DSM-5 Personality Disorders
SD	Standard deviation
SEM	Standard error of measurement
SPD	Standard error of the difference
START	Short-Term Assessment of Risk and Treatability
WHO	World Health Organization

Typographic conventions

Convention	Use in this book
Italic	Titles of works; emphasis; names of statistical tests
Bold	Headings and, within tables, the labels of rows requiring emphasis
En dash (–)	Ranges of numbers and pages; interruption of sense
Em dash (—)	Parenthetical interruption, used sparingly
[...]	Editorial notes; square brackets mark information that the publisher or author must supply
%	Used only in numeric expressions (for example, 16 per cent is written out)

Notes on usage

Titles of diagnostic classifications are given in full at first mention in each chapter, with the abbreviation thereafter. The names of instruments are

given in full at first mention, with the abbreviation thereafter; instruments are not abbreviated when the abbreviation would be unfamiliar to a general reader.

British spelling is used throughout, except in the titles of published instruments, organisations and works.

Preface

This book is written for doctoral students and practising psychologists who are learning to conduct psychological assessment at a level that can withstand scrutiny — by a supervisor, a multidisciplinary team, a tribunal, or a client who reads the report and asks, reasonably, how the conclusion was reached.

Its premise is that advanced assessment is distinguished less by the instruments used than by the quality of the reasoning that surrounds them. Any competent practitioner can administer a battery. What distinguishes advanced practice is the capacity to formulate a question that matters, to select methods proportionate to that question, to weigh evidence of uneven quality, to hold several explanations in view at once, and to communicate a conclusion that is both decisive enough to guide action and honest about its margins.

Three commitments organise the book.

Assessment exists to answer a question. Instruments, interviews and records are means; the decision the assessment was commissioned to inform is the end that gives them sense. Each chapter therefore begins from the question and works towards the method, rather than the reverse.

Every conclusion is probabilistic. Scores carry error, informants disagree, and diagnostic categories are useful simplifications of heterogeneous presentations. The book treats measurement error, base rates and diagnostic uncertainty not as technical footnotes but as the substance of responsible interpretation.

Assessment is an ethical act. Its conclusions can open or close access to treatment, education, employment, custody and liberty. The practitioner

therefore owes the client a transparent account of what was measured, what was inferred and what remains uncertain.

Organisation

The sixteen chapters are grouped into five parts. Part I establishes the foundations: the purposes of assessment, the design of interviews, and the psychometric basis of score interpretation. Part II addresses the assessment of dispositions and abilities — personality, cognitive function, and achievement, aptitude and career. Part III takes up the major clinical presentations: mood, anxiety, obsessive-compulsive, trauma-related, bipolar, psychotic, substance-related and enduring personality difficulties. Part IV considers children and adolescents, couples and families, and the cultural, linguistic and disability considerations that condition every interpretation. Part V addresses risk and safeguarding, and closes with the integration of evidence into formulation, reports and feedback.

Each chapter follows a consistent structure: learning objectives; key terms; an introduction that places the topic in the broader argument of the book; numbered sections that move from principle to application; an Advanced Case Conference that demonstrates the reasoning on a composite case; a summary; review and discussion questions; and a reading list. The case conferences are fictional composites constructed to illustrate assessment reasoning, not descriptions of identifiable clients.

A note on scope and limits

The book does not reproduce proprietary test items, scoring materials or copyrighted diagnostic criteria. Students must consult the authorised manuals of individual instruments and the current editions of the diagnostic classifications before administering or interpreting any measure. A textbook can teach principles; it cannot replace test-specific training, supervised practice or professional competence.

Diagnostic systems are revised. This edition refers to the *Diagnostic and Statistical Manual of Mental Disorders*, fifth edition, text revision (DSM-5-TR), together with approved DSM-5-TR updates available at the

time of writing; to the World Health Organization's *Clinical Descriptions and Diagnostic Requirements for ICD-11 Mental, Behavioural and Neurodevelopmental Disorders* (2024); and to the 2014 *Standards for Educational and Psychological Testing*. Practitioners remain responsible for checking later revisions and jurisdiction-specific requirements.

British spelling has been used throughout, except in the titles of published instruments, organisations and works, which retain their original spelling.

[AUTHOR TO SUPPLY: place and date of writing]

How to Use This Book

The book is designed for a doctoral course that combines seminars, supervised practice and independent reading. It can be read sequentially, but it is also organised so that it can be used flexibly.

Sequential reading

Chapters 1 to 4 establish the reasoning and measurement framework on which everything else depends. Readers who are new to assessment should read these chapters first, in order. Chapters 5 to 16 can then be read according to the curriculum, although each assumes familiarity with the concepts introduced in Part I.

Reading alongside a placement

Each chapter's Advanced Case Conference is a worked example of the reasoning the chapter describes. Readers on placement are encouraged to read the case conference alongside their own caseload, and to write their own case conference in the same format for supervision. The format — referral, evidence, reasoning, decision, teaching point — is deliberately the same as the structure of a well-written report.

Diagnostic systems

Two classification systems are referred to throughout: DSM-5-TR and ICD-11. They are not interchangeable, and the differences matter for some presentations, notably personality disorder, post-traumatic stress disorder and substance use. Readers should consult the primary texts for criteria,

and should note which system a given service uses, since the criteria applied affect eligibility for treatment and for funding.

Instruments

The book discusses instrument families and the principles by which they are evaluated, but it does not reproduce items, scoring rules or norm tables. When learning a specific instrument, readers should work from the manual, under supervision, with attention to the reliability, validity, norm group and stated limits of interpretation discussed in Chapter 3.

The learning features

Each chapter contains:

- **Learning objectives**, which state what the reader should be able to do after the chapter.
- **Key terms**, listed at the start and defined in the glossary.
- **Numbered sections**, which move from principle to application.
- **Tables**, which summarise distinctions that are easily confused.
- **Figures**, which present the chapter's conceptual structure.
- **An Advanced Case Conference**, which demonstrates the reasoning on a composite case.
- **A chapter summary**, which restates the argument.
- **Review questions**, which test comprehension and application.
- **Discussion questions**, which are designed for seminar use and have no single correct answer.
- **Further reading**, which identifies the sources on which the chapter draws.

A note for instructors

The discussion questions are written to expose disagreement rather than to elicit a correct answer, and they work best when students are asked to de-

fend a position and then to argue the opposite. The case conferences can be used as the basis for role-play, for written formulations, or for peer review exercises in which students critique each other's reasoning. Appendix A provides a planning and quality checklist that students can use as a self-assessment tool before submitting assessed work.